

Case Study

Reason for Services

Client is a 16-year old female, attending the clinic for trauma-based cognitive behavioral therapy. The intake session with the clinic was informative, both from their reports and the Caregivers' reports and assessments of emotion and functioning. Client reported being constantly worried, a negative cognition and mood, and difficulties with sleeping. They struggled to complete schoolwork and at-home chores, reporting some inability to focus on tasks. Caregivers reported that Client is intelligent, but lacks necessary motivation. They also reported that Client can easily become overwhelmed and anxious in regards to multiple tasks. They reported that Client tends to internalize feelings and avoids discussing them with Caregivers. They also reported the Client to be impulsive and revengeful, with a low tolerance for frustration. The intake assessments with Client met criteria for a DSM-5 diagnosis of Post-Traumatic Stress Disorder (PTSD), and Generalized Anxiety Disorder (GAD). These assessments ruled out the prior diagnoses of Reactive Attachment Disorder (RAD), Disruptive Mood Dysregulation Disorder (DMDD), and Autism Spectrum Disorder (ASD).

Treatment Objectives and Plan

Sessions began with weekly psychotherapy, specifically Trauma-Focused Cognitive Behavioral Therapy (TF-CBT), to help Client process childhood trauma. This would include a psycho-education component for Caregivers to understand the effects of childhood trauma, as well as effective parenting strategies for survivors of childhood trauma. The time spent with Caregivers would also allow for Motivational Interviewing in the case of concern or confusion relating to treatment.

Note. At session #12, it was determined that time with Caregivers needed to be extended, and a new format was created: 50 minutes of TF-CBT with Client, followed by 30 minutes of TF-CBT and Motivational Interviewing with Caregivers.

Upon completion of the TF-CBT, treatment will begin for Dialectical Behavioral Therapy (DBT) to improve Client's interpersonal skills within peer relations. A recommendation was also made following successful completion of DBT, for Client to be re-evaluated for remaining problems in case further treatment is necessary.

My Role in Treatment

During the scheduled client sessions, I sat in the observation room so as to not intrude on the client and clinician. I took de-identified notes so I could contribute in weekly meetings with supervisors, and ask the clinician any questions I had following the session. During my time observing this case, I made sure to keep updated on all necessary certifications and information, to ensure I was still in the scope of competence for completion of a case summary.